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The Indian Eldercare Playbook

A practical manual for adult children caring for ageing parents.

104m
Indians aged 60+

75%
with a chronic disease

Rs 5L
free cover at 70+

8.2%
on senior savings

INSIDE

The Ayushman card for everyone over 70, and the insurance rule that ended age refusal
What care costs in 2026, from a Rs 12,000 attendant to a Rs 1 lakh community
The 2007 Act, the Maintenance Tribunal, and the property clause that voids a transfer
The October 2026 pension rename, and the state top-ups that double it
Seven working templates: care agreement, attendant contract, safety audit, and more

FIRST EDITION · NOVEMBER 2026 · DIGITAL EDITION (PDF)

For readers in India

How to use this book

This is a manual, not a book to read once. It is arranged in the order a family usually hits each problem, and every chapter ends with what to do this week, because the useful unit of eldercare is the week, not the year.

Four ways to use it, depending on where you are today.

- **Nothing has happened yet, and you want to be ready.** Read chapters two and four this weekend, build the eldercare file, and check the Ayushman status. That is two hours of work that will save you weeks later.
- **Something just happened.** Turn to the 72-hour card on the next two pages, then chapter seven if it was a fall or a hospital admission.
- **You are managing care already and it is not working.** Start at chapter eight for the attendant problem and chapter thirteen for the family problem. Those two are where most arrangements fail.
- **You are planning money.** Chapters three, four, five and eleven, in that order. Together they are the difference between a family that funds care and a family that sells a house.

The three rules this book keeps returning to

1. **Match the care to the need, not to the budget.** Buying the wrong rung of the care ladder is the most expensive mistake in eldercare, in both directions.
2. **Claim everything before you spend anything.** Insurance, the 70+ card, the state pension, the free assistive devices. Families routinely pay for what they were already entitled to.
3. **Write it down.** Every dispute in this book, medical, financial or familial, gets easier with a document and harder without one.

A note on numbers and dates

Every figure here is sourced and the sources are listed in the appendix. Rates and scheme names change: the Senior Citizens' Savings Scheme rate is reset every quarter, state pensions change with state budgets, and one national pension programme was renamed in October 2026 while this edition was being prepared. Where a number is likely to move, the appendix tells you the primary document to check rather than a news summary of it.

The one thing to do before you turn the page

Open your phone and photograph your parent's Aadhaar, PAN, health insurance card and current medicine strips. Send them to yourself. That is the first page of the eldercare file, and it takes ninety seconds.

The 72-hour card

Print this. Fill in the blanks with a pen. Keep one copy in your wallet and one on the parent's fridge. It is written to be read by someone who is frightened and in a hurry.

If there is a fall, a collapse, chest pain or bleeding

1. Call 108 for an ambulance. Say the age, the address with a landmark, and what happened, in that order.
2. Do not move them if there is a suspected fracture, a head injury or back pain. Keep them warm and still.
3. If they are unconscious but breathing, turn them on their side. If they are not breathing, start chest compressions and keep going until help arrives.
4. Carry the medical sheet, the medicine strips and the insurance card. If you cannot find them, go anyway.
5. At the hospital, say the word 'cashless' at the insurance desk before you sign anything.

The numbers to write down now

	Fill in
Ambulance	108
National senior helpline	14567
Family doctor and phone	
Nearest hospital, with distance	
Insurance TPA toll-free	
Ayushman card number	
Sibling 1 and phone	
Sibling 2 and phone	
Neighbour or building contact	

The five sentences that work in a hospital

- 'What is the diagnosis in writing?'
- 'Is this admission cashless, and has pre-authorisation been filed?'
- 'What is not covered by the policy on this bill?'
- 'What is most likely to go wrong in the next two weeks, and what should bring us back immediately?'
- 'Who is the treating consultant, and when is the round tomorrow?'

What not to do in the first 72 hours

Do not sign a blank or partially filled insurance form. Do not pay the full estimate before the pre-authorisation decision. Do not move the parent to a second hospital because the first is inconvenient; continuity of care matters more than the room. And do not make any decision about selling property, changing a will or moving anyone into a facility while an acute episode is running. Nothing that feels urgent in hour 12 is still urgent in week 4.

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Fifteen chapters, one appendix of sources, and seven printable templates in chapter fourteen. The 72-hour card on page 3 is designed to be used in a panic and can be read in ninety seconds.

CHAPTER ONE

What you are actually walking into

One in five Indians will be over 60 by 2050. Almost none of the systems around them were built for that. Here is the shape of the problem, so the rest of this book has somewhere to stand.

Your father forgets to take his blood pressure tablet on Tuesday. By Thursday he has had a fall in the bathroom, and by Friday you are sitting in a hospital corridor in another city with a bag of his papers, no idea what his insurance covers, and a sibling on the phone asking what the doctor said. That is the moment this book is written for.

Not the sentimental version. The operational one. Eldercare in India is not a single decision; it is a hundred small administrative acts performed under emotional pressure, usually by one adult child, usually without training, usually while holding down a job. The system is fragmented: hospitals, insurers, municipal offices, pension desks, banks, siblings and the parent themself all hold a different piece. Nobody hands you the map.

The numbers that explain why this is suddenly everyone's problem

India had about 104 million people aged 60 or above in the mid-2020s, a little over 10 per cent of the population. NITI Aayog's 2024 paper on senior care projects that to reach 319 million, or 19.5 per cent of the country, by 2050. In absolute terms that is roughly the entire population of the United States, added inside one generation.

104m
Indians aged 60+ today, just over 10% of the population

319m
Projected 60+ population by 2050, 19.5% of India

75%
Of Indians over 60 live with at least one chronic disease

34%
Of the elderly are still working; most others are dependent

The health picture is worse than the demographic one. Roughly three in four older Indians live with at least one chronic condition, and a large share live with two or three at once. HelpAge India's 2026 survey of 2,224 older people across 20 rural districts in 10 states found that 46 per cent reported at least one long-term impairment affecting daily activities, most commonly mobility (32 per cent) and vision (24 per cent). Nearly half, 48 per cent, were financially dependent on others to some degree, and 92 per cent had a personal monthly income below Rs 10,000.

That last number is the one to hold on to. The typical Indian parent you are caring for has a small pension or none, some savings, a house, and a son or daughter who earns in a city. The care economy runs on that arrangement, and it is fragile.

Four gaps you will fall into, in this order

Almost every eldercare crisis in India is one of four failures. Knowing which one you are in tells you what to do next.

1. **The information gap.** Nobody has a single list of the parent's medicines, allergies, doctors, policies, accounts and documents. When an emergency happens, the family spends the first 48 hours reconstructing facts instead of making decisions.

2. **The money gap.** Families discover the cost of care only after they need it. A live-in attendant, a nurse, an assisted-living facility and a private hospital admission are four completely different budgets, and most households have planned for none of them.
3. **The entitlement gap.** The Ayushman card for the over-70s, the state pension top-up, the free assistive devices, the maintenance tribunal, the insurance rule that says an insurer cannot refuse your 74-year-old mother a policy: these exist, and most families never claim them because nobody told them.
4. **The family gap.** One child carries it. The others contribute opinions. Resentment builds quietly for years and then surfaces as a property dispute, which is the point at which the law becomes relevant.

What this book does differently

Most eldercare writing in India is either medical advice written for doctors, or inspirational content about respecting elders. Neither helps you on a Tuesday afternoon when the nurse has quit and the insurance pre-authorisation is stuck.

This book is organised as an operating manual. Fourteen chapters, each answering one question a family actually faces, in the order they usually face it. Every chapter ends with what to do this week. Every number is sourced from a government document, a regulator, a peer-reviewed study or a named industry source, and the edition date is on the cover because these figures move.

> **Read the next four pages first, even if you read nothing else**
> The 72-hour card on the following pages is the only part of this book designed to be used in a panic. Print it, fill in the blanks with a pen, and keep a copy in your wallet and one in your parent's home. Everything else in the book can wait until you can think.

A word about the word 'burden'

Policy documents describe India's ageing as a burden and a silver economy in the same paragraph. Both are true and neither is about your mother. What matters practically is that care has a cost, the cost is rising, and the person best placed to plan for it is you, now, while nothing is on fire. That is the entire thesis of this book.

One more thing before we start. Your parent is not a project. Every efficient thing in these pages, the files and the checklists and the scripts, exists so that you spend less of your time on administration and more of it on the part that only you can do. Keep that ratio in mind when the paperwork feels endless.

CHAPTER TWO

Week one: build the eldercare file

Before you fix anything, collect everything. One folder, one page, four categories. This is the highest-return two hours in the entire book.

The single most useful object you can create in eldercare is a file. Not a mental list, not a WhatsApp group, a physical folder plus a digital copy that any sibling can open at 2 a.m. from another city. Families that have one make better decisions faster; families that do not repeat the same argument every time a doctor asks a question.

Build it in four passes. Do not try to do it perfectly in one sitting; the medical pass takes an evening, the financial pass takes a week because it involves phone calls.

Pass one: identity and entitlement documents

Photograph every document, front and back, and store the images in one album shared with every sibling. Paper originals stay in the parent's home in a labelled envelope; a second set stays with you.

- Aadhaar card, PAN card, voter ID, ration card
- Passport (check the expiry date today; renewing later is far harder)
- Bank passbooks and the first page of each account, showing the account number, IFSC and the nomination field
- Pension Payment Order (PPO) for government pensioners, or the pension sanction letter from the employer
- Health insurance policy schedules, including any group cover still running from a former employer
- Ayushman card, if any, and the ABHA (Ayushman Bharat Health Account) number
- Property papers: sale deed, khata or 7/12 extract, municipal tax receipts
- Will, if one exists, and the name of the person holding it
- Driving licence, and the disability certificate or UDID if there is one

Pass two: the medical sheet

This is one A4 page, updated whenever something changes, and it is the document you hand to any doctor, at any hospital, in any city. Keep a printed copy in the parent's purse or wallet and a photo on your phone's lock screen.

Section	What goes on it	Why it matters
Conditions	Every diagnosis with the year it started	Doctors ask 'since when' more than 'what'
Medicines	Brand, dose, timing, who prescribed it, since when	Duplicate prescriptions are the commonest home error
Allergies	Drug, food, and any reaction seen before	Prevents a preventable reaction in an emergency
Doctors	Speciality, hospital, phone number	Referrals move faster when the name is known

Section	What goes on it	Why it matters
Baseline	Last BP, sugar, HbA1c, creatinine, weight, with dates	A single abnormal reading means little without a baseline
Implants	Stents, pacemaker, knee or hip replacement, with year and hospital	Determines what scans and procedures are safe
Insurance	Policy number, TPA name, toll-free number, sum insured	The first question at every admission desk

Two habits make this sheet work. First, date every entry. Second, when a medicine changes, strike through the old line rather than erasing it, because the history of what was tried is itself clinical information.

Pass three: the money map

You are not auditing your parent's finances. You are establishing four numbers, because every care decision later depends on them.

1. **Monthly inflow.** Pension, interest, rent, any family contribution. Add up the actual credits in the last three months of the bank statement.
2. **Monthly outflow.** Household expenses, medicines, help at home. Do not estimate; read the statement.
3. **Liquid reserve.** Money available within 48 hours without selling anything: savings accounts, fixed deposits, the sweep-in balance.
4. **Care ceiling.** The maximum monthly amount the family can commit to care without touching the house or a retirement corpus. Write it down as a number. This is the constraint every later choice is tested against.

> **The one question that changes everything**
>
> Ask, plainly: 'If you needed full-time help at home for a year, what would you want to spend, and from where?' Most parents have an answer and have never been asked. The answer tells you whether you are planning for a Rs 15,000-a-month arrangement or a Rs 60,000-a-month one, and that difference decides the rest of this book.

Pass four: the people list

Write down, with phone numbers: the family doctor, the chemist who actually knows the prescriptions, the neighbour or building watchman who checks in daily, the maid or cook, the local relative who can reach the house in twenty minutes, and the bank branch manager. In a crisis you will call these seven people in roughly that order, and you will not want to be searching for numbers.

Where to keep it

Three copies, three places: the original folder in the parent's home, a scanned set in a shared cloud folder that every sibling can access, and a one-page emergency card in your wallet. Do not use a document that only one sibling controls. The point of the file is that it removes you as a single point of failure.

Review it twice a year, on the parent's birthday and one other fixed date. Twenty minutes, twice a year, and the file never becomes archaeology.

CHAPTER THREE

What care actually costs, and how to fund it

Care is not one expense. It is a ladder with five rungs, and families usually pay for the wrong rung. Here are the 2026 numbers and the funding order that wastes the least.

The most expensive mistake in Indian eldercare is buying the wrong level of care. A family hires a 24-hour trained nurse for a parent who needs company and help with bathing. Another leaves a parent alone at night because an attendant 'is too expensive', and pays for three weeks of hospital after a fall. Cost is not the issue; matching is.

The care ladder

Rung	What it is	Typical monthly cost, 2026
1. Monitoring	Daily check-in calls, a neighbour, a wearable or phone SOS	Rs 0 - 2,000
2. Home help	Cook, cleaner, someone for shopping and medicines	Rs 5,000 - 15,000
3. Attendant	Live-in or 12-hour help with bathing, toileting, mobility	Rs 12,000 - 25,000
4. Home nursing	Trained nurse: wound care, injections, catheter, vitals	Rs 25,000 - 60,000
5. Facility	Assisted living, retirement home, nursing home	Rs 25,000 - 80,000+ (premium crosses Rs 1 lakh)

These are the ranges reported by home-care providers in 2026. Attendants on a live-in basis sit at Rs 12,000 to Rs 25,000 a month, with part-time hourly help at Rs 150 to Rs 300 an hour. Trained nurses run Rs 25,000 to Rs 60,000 a month, or Rs 300 to Rs 800 an hour. Assisted living is quoted at Rs 25,000 to Rs 80,000 a month including accommodation, meals, housekeeping and basic medical monitoring, with premium communities above Rs 1 lakh.

Metro rates sit at the top of every range. In Bangalore, providers were quoting roughly Rs 1,100 a day for an attendant and Rs 800 an hour for a nurse in 2026. Add 10 to 15 per cent a year and you have a planning assumption that has held for most of this decade.

The number families underestimate: the hospital episode

A single admission does more damage to an eldercare budget than a year of home help. A cardiac procedure, a hip fracture, a prolonged ICU stay or a course of cancer treatment can consume a family's entire liquid reserve in three weeks. This is why health cover, in the next chapter, is not optional housekeeping but the single largest financial lever you have.

Plan for the episode, not the average

A useful rule from insurance planning: your liquid reserve should cover the deductible, the non-payable items and the co-payment of one serious admission, plus three months of care costs. For most urban households that is Rs 1.5 lakh to Rs 3 lakh. If you do not have it, the priority is to build it before you upgrade the care rung.

The funding order that wastes the least money

Families tend to spend in the order that feels natural: their own savings first, then the parent's fixed deposits. The correct order is nearly the reverse, because several sources of money are use-it-or-lose-it.

1. **Insurance first.** Claim everything the policy allows before paying from savings. A year of unused cover does not roll over.
2. **Government entitlements second.** The Ayushman cover for the over-70s, state pension top-ups and free assistive devices cost nothing to claim and are covered in chapter eleven.
3. **The parent's income third.** Pension and interest should fund routine care before capital is touched.
4. **The parent's capital fourth.** Fixed deposits and the Senior Citizens' Savings Scheme, which paid 8.2 per cent for the October to December 2026 quarter and allows up to Rs 30 lakh per person.
5. **The children's income fifth,** as a monthly committed amount rather than ad hoc transfers, so it can be budgeted.
6. **The house last.** Property is the emergency reserve, not the first cheque. Selling or mortgaging it removes the option of ageing in place, which is usually what the parent wants most.

Building a care budget you can actually run

A workable eldercare budget has four lines and a contingency. Keep them separate in a spreadsheet, because they behave differently.

Line	What it covers	Behaviour
Fixed care	Attendant or nurse salary, agency fee, facility charges	Rises in steps, renegotiate annually
Medical recurring	Medicines, tests, physiotherapy, doctor visits	Rises 8-12% a year; the most predictable inflation
Household	Rent, utilities, groceries, transport	Stable, but rises if care staff live in
Contingency	One admission, equipment, an emergency trip	Target Rs 1.5-3 lakh, held in a sweep FD

Review it every quarter against actual bank debits, not against memory. The gap between what families think they spend on a parent and what they actually spend is routinely 30 per cent, always in the same direction.

Three households, three budgets

Abstract ranges do not help anyone commit. Here are three worked cases, built from the 2026 cost ranges above, to show what the same decision looks like at three levels of need. Names are illustrative; the arithmetic is not.

Line	A: independent, 68	B: needs help, 78	C: high needs, 84
Home help or attendant	Rs 0	Rs 18,000 (12-hour attendant)	Rs 42,000 (nursing + night attendant)
Medicines and tests	Rs 3,500	Rs 6,500	Rs 9,000
Physiotherapy or specialist visits	Rs 0	Rs 3,000	Rs 6,000
Household and utilities	Rs 12,000	Rs 14,000	Rs 15,000
Transport to appointments	Rs 1,500	Rs 2,500	Rs 3,500
Monthly total	Rs 17,000	Rs 44,000	Rs 75,500
Annual total	Rs 2.04 lakh	Rs 5.28 lakh	Rs 9.06 lakh

Line	A: independent, 68	B: needs help, 78	C: high needs, 84
Funded by	Pension and interest	Pension plus one child's transfer	Pension, children, and capital drawdown

Two things are worth noticing. Case B, the most common one, costs about Rs 5.3 lakh a year, which is roughly the cost of one private-college year and is entirely invisible until you add it up. And in case C the annual figure crosses Rs 9 lakh, which is why the contingency line matters: one hospital admission at that level is a second annual budget.

Build your own version of this table this week, using the parent's actual bank debits for the last three months as the base for every line except the care line. Then add the care line from the quote you actually have, not from the average. The gap between the two is your planning problem, stated as a number.

When siblings share the cost

Money between siblings is where eldercare turns into a family dispute. Two rules prevent most of it. First, contributions are either equal or proportional to income, agreed once, in writing, with a number and a date. Second, the sibling who provides hands-on care is contributing, and that contribution has a monetary value equal to the attendant wages they are saving the family. A template for this agreement is in chapter fourteen.

If one sibling cannot pay, say so explicitly and substitute something specific: the quarterly hospital run, the insurance paperwork, the weekend shift. Vague non-participation is what corrodes; named, bounded non-participation usually does not.

CHAPTER FOUR

Health cover: the 70+ card and the policy you can still buy

Two things changed the arithmetic for older Indians: free cover for everyone over 70, and a regulator's rule that insurers must sell to all ages. Most families use neither.

Health insurance for the elderly in India used to be a closed door: buy before 60 or pay for everything yourself. That door has been opened twice, from two directions, and both openings are still underused.

The first opening: free cover for everyone aged 70 and above

Ayushman Bharat PM-JAY was extended to all senior citizens aged 70 and above, irrespective of income, with cover of Rs 5 lakh per year. The eligibility criterion is a single line: the person must be 70 or older as recorded in Aadhaar. There is no income test and no family-size test.

- Approximately 6 crore senior citizens, across about 4.5 crore families, are 70 or above and therefore eligible.
- Aadhaar-based e-KYC is mandatory; Aadhaar is the only document needed to enrol.
- Where Aadhaar records only a birth year, the age is determined from that year, so a person whose Aadhaar shows 1956 qualifies in 2026.
- Enrolment issues the Ayushman Vay Vandana card, which is what you show at a hospital counter.
- Where both husband and wife are 70 or above, each is entitled to a separate cover of Rs 5 lakh rather than sharing one.

Enrolment is done at an empanelled hospital's Ayushman counter, at a Common Service Centre, or through the Ayushman Bharat beneficiary portal. Take the parent's Aadhaar and a mobile number linked to it. The card is usually generated the same day.

What the 70+ cover does not do

It pays for hospitalisation at empanelled hospitals, not for outpatient consultations, most medicines, home care or diagnostics done outside admission. It does not replace a private policy; it sits underneath one. Families who cancel a private policy because 'the government now covers it' usually regret it within a year, because the private policy pays for the pre- and post-hospitalisation window that PM-JAY largely leaves to you. Keep both.

The second opening: insurers must sell to your 74-year-old

IRDAI's master circular of 29 May 2024 changed the rules of the market. Insurers must now offer their products to all ages, and to prospects with pre-existing diseases and medical conditions. Two other changes matter as much as the age rule.

Rule	What it means for your parent
All ages must be offered cover	Refusing a policy solely because the applicant is 72 is no longer permitted; underwriting may load the premium instead
Pre-existing disease waiting period cut to 3 years	From 4 years. Conditions declared at purchase are covered after 36 months

Rule	What it means for your parent
Moratorium period cut to 5 years	After 60 continuous months, a claim cannot be contested except for proven fraud, misrepresentation or non-disclosure
Cashless and pre-authorisation discipline	Insurers must decide cashless requests within defined timelines; ask for the turnaround in writing at admission

The practical consequence: if your parent is 72 with diabetes and hypertension, you can now buy a policy. It will be expensive and it may carry loading, but it exists, and the three-year clock on pre-existing conditions starts the day you buy. Every year you delay is a year added to the wait before the conditions that will actually cause the claim are covered.

How to choose, in the order that matters

1. **Check what already exists.** A group cover from a former employer, a policy bought at 55 and forgotten, a corporate floater that includes parents. Dig out every schedule before buying anything.
2. **Size the sum insured to the episode, not the premium.** In metros, Rs 10 lakh is the realistic floor for a household with a 75-year-old. Rs 5 lakh is exhausted by one cardiac admission.
3. **Prefer a dedicated senior-citizen product** over a family floater. Senior products price the risk honestly instead of loading it onto everyone, and usually have fewer sub-limits on joint replacements and cataract.
4. **Read the sub-limits and the room-rent cap.** A policy with a 1 per cent room-rent cap per day will not cover a metro ICU. These clauses, not the premium, decide the claim.
5. **Check the co-payment and the deductible.** A 20 per cent co-pay on a Rs 8 lakh bill is Rs 1.6 lakh from your pocket at the worst moment.
6. **Check the network hospitals near the parent's home,** not near yours. Cashless at a hospital 40 km away is useless at 2 a.m.
7. **Buy before the next birthday.** Premiums step up by age band; a policy bought at 68 is materially cheaper for life than the same policy at 71.

If the parent is already 80, or already sick

Two fallbacks remain. A senior citizen policy with a higher loading and a longer waiting period is still worth buying if the alternative is nothing. And a hospital cash or fixed-benefit policy, which pays a set amount per day of admission regardless of the bill, will not cover a large claim but does cover the incidental costs that drain a household: attendant charges, transport, the loss of income while you sit in the ward.

Do not buy a critical illness policy for an 80-year-old. The premiums are high, the survival and waiting conditions are strict, and the same money put into a dedicated medical contingency fund does more work.

Claim discipline: the part nobody teaches

Most rejected claims are rejected on documentation, not on merits. Three habits prevent it. Get pre-authorisation before a planned admission, not at the counter on the morning of surgery. Keep every discharge summary, investigation report and pharmacy bill in the eldercare file, because reimbursement claims are decided on paper. And when a claim is rejected, escalate in writing to the insurer's grievance redressal officer, then to IRDAI's Bima Bharosa portal; the escalation itself resolves a meaningful share of disputes.

Finally, register an ABHA number for the parent. It links their health records across hospitals, which means the next doctor can see the last discharge summary without you carrying a plastic bag of paper.

CHAPTER FIVE

The tax and pension arithmetic

You are probably leaving Rs 40,000 to Rs 1.5 lakh a year on the table, and part of it is now unavailable because of the new tax regime. Here is the exact map, under the Income-tax Act 2025.

From the 2026-27 tax year, deductions are governed by the Income-tax Act 2025, which replaced the 1961 Act and renumbered every section you know. The amounts did not change. The section numbers did, and a lot of advice still floating around quotes the old ones.

The bigger issue is the regime. Under the new tax regime, which is now the default, most of the senior-citizen deductions below are not available at all. That single fact changes the arithmetic for a household deciding whether to claim parents as dependents for tax purposes.

The renumbered map for senior citizens

Benefit	Old section	New section	Limit	In new regime?
Interest income on deposits	80TTB	153	Rs 50,000 a year for a senior citizen	No
Health insurance premium and medical expenditure	80D	126	Rs 50,000 for a senior citizen, including Rs 5,000 preventive check-up	No
Treatment of specified diseases	80DDB	128	Rs 1,00,000 for a senior citizen, on a specialist's prescription	No
Senior Citizens' Savings Scheme deposit	80C	123	Within the Rs 1.5 lakh overall cap	No

Two things follow. First, if the parent has taxable income and is on the old regime, section 153 alone is worth up to Rs 50,000 of interest income tax-free, which at the 20 per cent slab is Rs 10,400 a year of tax not paid, for filling in one line. Second, if you pay the parent's premium and they are on the new regime, the deduction is gone and the premium is simply a cost.

The TDS change that catches people out

Forms 15G and 15H, the declarations that stop a bank deducting TDS on interest, are replaced by a single Form 121 under the new Act. If your parent has been submitting 15H at the branch for years, that habit needs updating in the 2026-27 year, or the bank will deduct TDS and the refund will arrive a year later.

Pension income: the parts that are not taxed

Retirement benefits are taxed in three different ways and families routinely overpay because they treat all of it as income.

- Commuted pension: the lump sum received on commutation is exempt to the extent allowed for the category of employee, with the balance taxable as salary in the year of receipt.
- Gratuity: exempt up to the statutory ceiling for government employees and within the prescribed limit for others.
- Leave encashment on retirement: exempt within the prescribed limit.

- Family pension: taxed as income from other sources, with a standard deduction of one-third of the pension or Rs 15,000, whichever is lower.

If the parent retired decades ago, most of this is history. It matters in one situation: a parent who retires during the years you are caring for them, where the commutation and gratuity decisions are made once and are hard to undo.

The Senior Citizens' Savings Scheme, honestly assessed

SCSS paid 8.2 per cent per annum for the October to December 2026 quarter, unchanged for the fourteenth consecutive quarter and held at that level since April 2023. It is the highest rate in the small savings basket and it is sovereign-backed, which is why it is the default home for retirement capital in most Indian households.

Feature	Rule
Eligibility	Age 60+, or 55-59 if retired under superannuation or VRS, or 50+ for retired defence personnel
Deposit	Rs 1,000 minimum, Rs 30 lakh maximum per person across accounts
Tenure	5 years, extendable once by 3 years
Interest	Quarterly, credited on the first working day of April, July, October and January
Joint account	Allowed with a spouse only
Early closure	Before 1 year: no interest. 1-2 years: 1.5% deducted. 2-5 years: 1% deducted
Tax	Interest fully taxable at slab; deposit qualifies under section 123

At the Rs 30 lakh ceiling, 8.2 per cent pays Rs 61,500 a quarter, or about Rs 20,500 a month. That is the single most useful number in retirement planning for a couple with a house and no employer pension: it converts a lump sum into a predictable monthly income with no market risk.

Where SCSS is the wrong answer

It is the wrong home for money you may need for a medical emergency within a year, because early closure costs 1 to 1.5 per cent. Keep the contingency in a sweep-in fixed deposit instead. And it is the wrong answer for a parent whose income is already above the taxable threshold and who is on the old regime without using section 153, because the interest is taxed at slab and the effective return falls sharply.

What to do this week

1. Find out which tax regime the parent filed under last year. This single fact decides whether the four deductions above are worth anything.
2. If they are on the old regime and have deposit interest, submit Form 121 at every bank branch where they hold a fixed deposit.
3. Check whether the parent's health premium is being paid from your account or theirs, and whether anyone is claiming section 126 for it.
4. If the parent has Rs 5 lakh or more in fixed deposits earning below 7 per cent, price out SCSS at 8.2 per cent before the next renewal.

CHAPTER SIX

Managing the parent's health day to day

Three in four older Indians live with a chronic condition. The work of managing it is unglamorous, repetitive and mostly done by whoever notices. Here is how to make it a system instead of a worry.

Chronic care for an older parent is not a series of crises. It is a handful of small loops, repeated: the medicine loop, the measurement loop, the appointment loop, the nutrition loop. When the loops run, crises get rarer. When they break, they break quietly and then all at once.

The medicine loop, which fails first

The commonest home medication error in Indian households is duplication: two doctors, two prescriptions, the same molecule under two brand names. The second commonest is silent discontinuation, where a medicine is stopped because the strip ran out and nobody reordered it.

1. **One list, one authority.** The medical sheet from chapter two is the only list. Any doctor adding a medicine writes on it or tells you to.
2. **Reconcile at every visit.** Carry the physical strips, not just the list, to appointments. Doctors spot duplication in seconds when the boxes are on the table.
3. **Ask for generic names.** Write the molecule next to the brand. It makes substitution at the chemist safe and it makes a second opinion possible.
4. **Use one chemist.** A chemist who knows the household catches duplication, keeps a running ledger and delivers. This is worth paying a small premium for.
5. **Fix the pill-box problem.** If the parent takes more than four medicines a day, use a weekly organiser filled by one named person on one named day. Do not rely on memory for anything taken twice daily.

The adherence question that actually works

Do not ask 'are you taking your medicines?' Everyone says yes. Ask: 'Show me yesterday's strip.' The number of tablets missing from a strip is the only honest measure of adherence you will get, and it takes ten seconds.

The measurement loop

Home measurement is only useful if it is recorded and dated. A BP reading in a WhatsApp message with no date and no time is noise; a reading entered on a card that the doctor can see is data.

Measure	How often	What the doctor needs to see
Blood pressure	Twice weekly, or daily if recently changed	Numbers with date and time, sitting, after five minutes rest
Weight	Weekly	A 2 kg loss in a month with no dieting is a finding
Fasting sugar	As advised, usually weekly to fortnightly	Fasting and post-meal, not one or the other
HbA1c	Every 3 months if diabetic	The trend, not the last value
Creatinine	Every 6-12 months, more often on multiple medicines	Because several common drugs are cleared by the kidney

The appointment loop

Fragmented care is the norm: a physician, a cardiologist, an orthopaedic, an ophthalmologist, a dentist, each seeing one organ and none seeing the person. You are the integrator. Four practices make that job tractable.

- Consolidate appointments into one week per quarter where possible. Travel and waiting are the real cost of a hospital visit for an 80-year-old, not the consultation fee.
- Always carry the medical sheet and the last three discharge summaries.
- Write down the three questions you actually need answered before you enter, and ask them in the first two minutes. Consultations compress.
- Ask one closing question every time: 'What would make you want to see them again sooner than the next appointment?' It surfaces the red flags the doctor would otherwise assume you know.

Nutrition and the protein problem

The most under-diagnosed nutritional issue in older Indians is not calorie deficiency but protein deficiency, which shows up as muscle loss, slow wound healing and a fall that would not have happened five years earlier. A diet of rice, dal and one vegetable, eaten out of habit and cooked for convenience, is usually adequate in calories and short on protein.

Practical fixes that work in Indian kitchens: add a second serving of dal or a bowl of curd at lunch; keep roasted chana or peanuts where the parent sits; use paneer, soya granules or eggs at breakfast two or three times a week; and check the teeth before assuming the parent is 'not hungry'. Untreated dental pain is a very common cause of quiet under-eating.

Also check hydration deliberately. Older adults have a blunted thirst signal, and a urinary infection from low fluid intake is one of the commonest reasons an elderly parent becomes suddenly confused.

Vaccination: cheap insurance people skip

Adult vaccination is poorly used in India. For a parent over 65, the conversation with their doctor should cover influenza (annual), pneumococcal disease, and shingles where available and affordable, plus tetanus and any travel-related vaccine. None of these is expensive relative to the admission they prevent.

Mental health, which nobody asks about

Depression in older Indians is common, under-recognised, and frequently mistaken for 'old age'. The tell is a change: a parent who stopped going to the temple, stopped calling the grandchildren, stopped caring about what they wear. That is not ageing; that is a symptom.

Ask about sleep, appetite and interest, in that order, and take a positive answer to a doctor rather than treating it as a mood. Two other things deserve a mention because they are missed constantly: hearing loss, which looks like confusion and withdrawal and is fixed with a hearing aid, and untreated pain, usually knees or back, which produces irritability that families read as personality change.

The six conditions you will actually manage

Most older Indian parents live with two or three of the same conditions. You are not treating them; the doctor does that. You are running the day-to-day layer that decides whether the treatment works. Here is what that layer looks like, and the one red flag in each that means call someone today.

Condition	What you actually do	Red flag: act today
Type 2 diabetes	Same meal times daily; fasting and post-meal readings on a dated card; foot check weekly for cuts and numbness; HbA1c every three months	Sugar below 70 with sweating or confusion; a foot wound that is not healing
High blood pressure	Reading twice weekly, sitting, after five minutes rest; salt reduction in cooking, not just at the table; watch for dizziness on standing	A reading above 180/110; sudden severe headache, chest pain, or one-sided weakness
Knee or hip osteoarthritis	Keep walking on level ground; sit-to-stand practice daily; weight reduction if possible; a stick in the opposite hand; avoid deep squats and stairs where avoidable	A joint that is hot, swollen and painful at rest, or sudden inability to bear weight
COPD or asthma	Inhaler technique checked by a professional, not assumed; avoid smoke, incense and dust; flu vaccination annually; keep a spare inhaler	Breathlessness at rest, bluish lips, or confusion during an attack
Vision: cataract and glaucoma	Annual eye check; a second pair of glasses as backup; night lights; large-print phone settings	Sudden vision loss, floaters with flashes, or eye pain with redness
Urinary and prostate problems	Reduce fluids after 8 p.m.; keep a clear path to the toilet at night; treat constipation, which worsens urinary symptoms	Inability to pass urine at all, or fever with burning and confusion, which in an older person usually means infection

Three cross-cutting principles apply to all six. First, one change at a time: if you alter diet, medicines and routine in the same week, you cannot tell what worked. Second, the parent's own report is the least reliable data you have, not because they lie but because older adults normalise decline; look at the numbers and the strips. Third, a sudden change in function, confusion, a fall, or loss of appetite in an older person is an infection until proven otherwise, most often urinary or respiratory, and it needs a doctor rather than a wait.

The home medicine cabinet and equipment list

A small, well-chosen set prevents most middle-of-the-night emergencies. Keep it in one box, in one place, and check expiry dates twice a year.

Item	Why	Note
Thermometer and BP monitor	The two readings you will be asked for	A validated upper-arm cuff, not a wrist one
Pulse oximeter	Distinguishes breathlessness from panic	Cheap and reliable if the fingers are warm
Glucometer with strips	For diabetics, and for any confusion episode	Check strip expiry; expired strips read falsely
Digital weighing scale	Unexplained weight loss is a finding	Weigh weekly, same time, same clothes
First-aid basics	Cuts, grazes, minor burns	Antiseptic, gauze, adhesive dressing, crepe bandage
ORS sachets	Dehydration from heat, diarrhoea or fever	Especially important in summer and during infections
Walking stick, walker, commode chair	Mobility and toileting safety	Check the rubber tip on the stick every month
Grab bars and anti-skid mat	The bathroom, which is where falls happen	Chapter nine has the full retrofit list
Spare spectacles and hearing-aid batteries	Cheap redundancy that prevents a bad week	Keep in the same box

Do not build a home pharmacy. Keeping leftover antibiotics, old painkillers and expired tablets 'just in case' is how older people end up taking the wrong thing at 2 a.m. Anything prescription-only should come from a doctor in the current month, not from a drawer.

What to do this week

1. Do the strip audit: look at every medicine box in the house, write the molecule name beside each brand, and check for duplicates.
2. Buy a weekly pill organiser and name the person who fills it.
3. Start a dated measurement card and stick it on the fridge.
4. Book one consolidated appointment week for the quarter, rather than scattering visits.
5. Ask the doctor directly about vaccination and about depression screening.
6. Check the expiry dates on everything in the medicine box and throw out what has gone.

CHAPTER SEVEN

Falls, dementia and the hospital

These three account for most eldercare emergencies. Each has a playbook, and each is far cheaper to prevent than to manage.

If you only prepare for three things, prepare for these. A fall, a cognitive decline that goes unnoticed until it is advanced, and an unplanned admission are the three events that change a family's year.

Falls: the event that ends independence

A hip fracture in an 80-year-old is not an inconvenience; it is a trajectory change. Roughly half of older people who fracture a hip lose some degree of independent living afterwards, and the fear of falling that follows reduces activity, which accelerates the weakness that causes the next fall.

Falls are not random. They cluster around four causes, and three of them are fixable in a weekend.

Cause	What to check	Fix
The bathroom	No grab bar, slippery floor, no seat in the shower, high threshold	Grab bars by the toilet and shower, anti-skid mat, plastic stool, threshold strip
Medication	Sedatives, BP medicines causing dizziness on standing, multiple drugs	Ask the doctor for a medication review specifically for fall risk
Vision and feet	Uncorrected vision, cataract, wrong spectacles, slippery footwear	Eye test, and footwear with a proper sole and back strap, never loose slippers
Weakness	Difficulty rising from a chair without using the arms	Daily sit-to-stand practice and a referral for physiotherapy

Two structural additions matter more than anything else: a light that can be switched on from the bed, and a clear path from bed to toilet with no furniture, no rug and no cable in it. Most night-time falls happen on that route.

What to do in the first ten minutes after a fall

Do not lift them. Ask them to move each limb; if a limb cannot move, if there is shortening or deformity, or if there is head injury, call an ambulance and keep them still and warm. If they can move everything and there is no head strike, help them to a chair slowly and watch for 24 hours. Any head injury on a blood thinner, or any loss of consciousness, means a hospital visit even if they seem fine, because intracranial bleeding in the elderly can present hours later.

Dementia: recognising it early enough to plan

About 8.8 million Indians over 60 live with dementia, a prevalence of 7.4 per cent in that age group, according to the national estimates derived from the Longitudinal Ageing Study in India. That is roughly one in thirteen people over 60, and the vast majority are undiagnosed.

The distinction that matters at home is between normal ageing and disease. Forgetting where the keys are is ageing. Forgetting what keys are for, or forgetting that the kitchen exists, is not. The practical markers are these:

- Repeating the same question within minutes, and not remembering the answer.
- Getting lost on a route walked for thirty years.

- Difficulty with money: paying the same bill twice, or being unable to count change.
- Personality change: suspicion, apathy, or sudden irritability in someone who was gentle.
- Difficulty with a task that was automatic: cooking a familiar dish, operating the washing machine, writing a cheque.

If two or more of these are present, get a cognitive assessment. Ask for a memory clinic or a neurologist or psychiatrist with geriatric experience. The point of a diagnosis is not the label; it is that it unlocks treatment for some causes, rules out reversible ones (thyroid, B12, depression, medication side effects, and a chronic subdural bleed after an old head injury), and starts the planning clock.

What changes at home after a dementia diagnosis

1. **Safety first.** Lock or disable the gas stove, remove the car keys, put a bell on the door, and register the parent's address and photograph with the local police station and with Elderline 14567.
2. **Simplify the environment.** Fewer objects, more labels, one place for each thing. Clutter reads as noise.
3. **Write things down.** A whiteboard by the door with the day, the date and who is coming today reduces repetitive questioning.
4. **Fix the routine.** Meals, walks and sleep at the same time daily. Predictability substitutes for memory.
5. **Money safeguards.** Add a joint holder or a mandate on accounts, set transaction alerts on the parent's phone to your number, and reduce the daily withdrawal limit. Financial exploitation of people with dementia is common and usually by someone known to the family.
6. **Tell the neighbours.** A short, factual sentence to two neighbours prevents a wandering incident from becoming a search.

The caregiver side of dementia is its own problem, and it is the reason families break. Dementia care is 24-hour supervision with no off switch, and one person cannot do it alone for years. Plan respite from the beginning: a day-care centre, a rotating family shift, or a paid attendant for a fixed block each week. Chapter thirteen deals with the caregiver's own limits directly.

The unplanned admission: what to carry and what to ask

Hospitals run on documents. The family that arrives with a folder is admitted faster, questioned less, and discharged with a correct bill.

Carry	Ask on admission	Ask before discharge
Aadhaar, insurance card, Ayushman card	Is this cashless, and has pre-authorisation been filed?	What is not covered by the policy on this bill?
Medical sheet and last discharge summary	Who is the treating consultant, and when is the round?	What is the diagnosis in writing, and what changed?
List of current medicines with strips	Are any home medicines to be stopped in hospital?	Which home medicines continue, which stop, which are new?
Advance directive, if one exists	Is a geriatric or palliative consult available?	What are the warning signs that mean come back immediately?

Two questions families forget and later regret. The first is for the discharge desk: 'What will this cost in total, and what is the estimate in writing?' The second is for the doctor: 'If we take them home, what is the one thing most likely to go wrong in the next two weeks?' Both answers change what you do next.

Also ask for the discharge summary before you leave the building, and check that the medicine list on it matches what you were actually handed. Mismatches at discharge are a leading cause of readmission within thirty days.

CHAPTER EIGHT

Hiring care at home

The attendant decision is the one families get wrong most often, and the one with the largest effect on the parent's daily life. Treat it like a hire, because it is one.

Most families hire an attendant in a hurry, from a reference, after a crisis. Then they discover in month two that the person cannot manage the parent's transfers, or is on the phone all night, or has quietly become the household's accountant. The fix is process, not luck.

First, decide the level, honestly

The care ladder from chapter three is the map. The question to answer is not 'how much can we afford' but 'what does the parent actually need help with'. Write the list. Bathing, toileting, dressing, walking to the bathroom at night, cooking, medicines, company, wound care, injections, catheter, oxygen.

If the list contains wound care, injections, a catheter, a feeding tube or oxygen, you need a nurse, not an attendant. An attendant doing nursing tasks is both a clinical risk and, if something goes wrong, a liability question with no good answer.

Agency or direct hire

	Agency	Direct hire
Cost	15-30% higher; the agency takes a margin	Lower monthly cost
Replacement	Usually within days, at no extra charge	You start the search again
Verification	Agency claims police verification and training	You verify yourself
Control	Less; the agency sets rules and rotates staff	More; you set terms
Best for	First hire, nursing tasks, families in another city	A known, referred person for a stable, low-medical need

For a first hire, use an agency. The replacement guarantee is worth more than the margin, because the first attendant often does not work out and you will not want to run a second search in three weeks.

The nine questions that separate candidates

1. 'What did you do for your last patient, on a normal day?' Listen for specifics about transfers and toileting, not adjectives.
2. 'Have you handled a patient who cannot stand without help?' Ask them to demonstrate the transfer, on you, in the room. This is the single most revealing test.
3. 'What would you do if you found them on the floor?' A good answer is calm, ordered and does not involve lifting.
4. 'How do you remember the medicines?' A good answer involves a box or a written list, never memory.
5. 'What did you do when the last family annoyed you?' Watch for a person who can describe a conflict without contempt.
6. 'Can you stay awake at night?' If night cover is needed, ask directly. Many attendants sleep.

7 'How many days a month will you need off, and who covers you?'

8 'Do you have a phone, and will you keep it away during care tasks?'

9 'What do you want to be doing in two years?' A person with a plan stays longer.

Verification, in the order that matters

- Aadhaar, with the name and address matching what they told you.
- Police verification certificate. If the agency cannot produce one, get it done yourself at the local police station; it is inexpensive and it is the single best filter.
- Two references from previous families, and call both. Ask the previous family what they would change, not whether they were happy.
- A health check, including a chest X-ray and, where relevant, a vaccination history. You are bringing someone into daily contact with a frail person.
- A photograph, kept in the eldercare file and shared with siblings.

The terms that prevent disputes later

Put the arrangement in one page, signed, in a language the attendant reads. It is not a legal formality; it is the thing you point to when memory differs. A template is in chapter fourteen. The clauses that matter:

- Monthly salary, the date it is paid, and what it does and does not include (food, accommodation, travel).
- Working hours and the night arrangement, stated explicitly.
- Days off per month and the notice period on both sides.
- The list of care tasks, written down.
- A rule that any medicine change comes only from the family or a doctor.
- A rule that no money is to be handled for the parent without the family being told, and no bank card or UPI access.
- Emergency contacts, and the instruction to call an ambulance first and the family second.

The one control that matters most

Never give an attendant access to the parent's bank account, UPI, card or pension. Financial abuse of older people in India is overwhelmingly committed by someone with legitimate physical access, and it is very hard to unwind. Pay the attendant directly, in a traceable transfer, and keep the parent's money separate.

Managing the arrangement after month one

The failure mode is drift: the attendant starts doing the shopping, then the banking, then deciding who visits. Prevent it with a short, boring ritual. A ten-minute call at the same time each week with the same three questions: what changed, what is running out, what do you need. And a monthly in-person or video check with the parent alone, where you ask how things are going when the attendant is not in the room.

Rotate the person who does the check if there are siblings. A single monitor gets managed by the person they are monitoring; two people who compare notes do not.

Caring from another city

If you live in a different city, you are not less able to care; you are managing at a distance, which is a different job. Four things make it work: a local named person who can be at the house in twenty minutes; a video call at a fixed time, not on request; a smart doorbell or a simple camera in the living room with the parent's knowledge and consent; and one sibling or cousin who holds the local paperwork, so documents do not have to travel.

Also register the parent on Elderline 14567, the national helpline for senior citizens. It handles information, guidance, emotional support and field intervention, and it is free.

CHAPTER NINE

Housing: ageing in place, or moving out

The house is usually the parent's last piece of independence. Retrofit it before you consider replacing it, and know exactly what you are signing if you do replace it.

Indian families default to ageing in place, and mostly they are right. Familiarity is therapeutic for older adults, and a move at 78 costs more in orientation and confidence than families expect. But ageing in place only works if the house is modified, and modification is usually cheaper than people assume.

The retrofit list, in order of return on money

Change	Typical cost	What it prevents
Grab bars in bathroom and by the toilet	Rs 1,500 - 4,000 each	The commonest fall location in the house
Anti-skid mat and a plastic shower stool	Rs 800 - 2,500	Slipping in the bathroom
A light reachable from the bed, and a night path light	Rs 500 - 2,000	Night-time falls on the bed-to-toilet route
Raised toilet seat or a commode chair	Rs 1,500 - 6,000	Squatting and standing, which is where knees fail
Remove or fix loose rugs, cables and low furniture	Rs 0	Trips, at no cost
A bed rail or a firm mattress	Rs 2,000 - 8,000	Rolling out of bed, and difficulty getting up
Non-slip footwear with a back strap	Rs 500 - 1,500	Slipper-related slips, which are surprisingly common
Stair rail on both sides, if there are stairs	Rs 3,000 - 10,000	Stair falls, which are the most injurious

A full retrofit of a normal Indian flat costs Rs 15,000 to Rs 40,000 and takes a weekend. That is less than one week of a private hospital bed in a metro. It is the highest-return spending in this entire book.

Two structural questions deserve a professional opinion rather than a handyman: whether the bathroom can be converted to a walk-in shower with a drain slope, and whether a ground-floor room can be made into a bedroom if stairs become impossible. Both are easier to do now than during a post-fracture discharge.

When ageing in place stops working

Four signals, in ascending order of seriousness. Care needs exceed what one attendant can safely provide. The parent is alone for long stretches and cannot reliably summon help. There is cognitive decline with wandering or stove use. And the family's own health is failing under the load. If two of the four are present, start looking at alternatives before a crisis chooses for you.

Assisted living and retirement homes: the honest assessment

The market has matured quickly and the quality range is enormous. Costs reported in 2026 run from Rs 25,000 to Rs 80,000 a month for assisted living that includes accommodation, meals, housekeeping and basic medical monitoring, with premium communities crossing Rs 1 lakh. Nursing homes and medical care centres sit above that.

What you are buying is not a room. It is a bundle: staff ratios, medical escalation, food, social contact and a contract. The contract is where families lose money.

The twelve-point contract check

1. What exactly is included, itemised, and what is charged extra (physiotherapy, doctor visits, diapers, attendant hours, ambulance).
2. The exit clause: notice period, refund of the security deposit, and the timeline for that refund in writing.
3. What happens if the resident's care needs increase; can they be asked to leave, and on what grounds.
4. Staff ratio by shift, and the qualification of the night staff.
5. The medical escalation protocol: which hospital, how far, who decides, who pays for the ambulance.
6. Whether a family member may stay overnight in an emergency.
7. Visitor rules, and whether the resident can leave freely.
8. The grievance mechanism and who is accountable.
9. Registered entity name, and the licences it holds.
10. Whether the facility has a written restraint policy. There should be none, in writing.
11. Insurance and liability for injury on the premises.
12. Food: who plans it, and whether dietary restrictions for diabetes or renal disease are actually handled.

The move itself

A move at 75 or 80 is a bereavement in miniature. The parent is losing a house, a neighbourhood, a set of familiar routes and often a spouse's memory attached to a room. Three things reduce the damage.

- Move the furniture that matters. The chair, the cot, the photographs, the prayer corner. Continuity of objects substitutes for continuity of place.
- Do a two-week trial stay before committing, if the facility allows it. Most do, and it changes the decision more often than families expect.
- Set the visiting rhythm before the move, not after. A fixed weekly visit on a fixed day is worth more than frequent unpredictable ones.

And keep the old house for six months if you can afford to. Reversing a move is far easier than reversing a sale.

If the house has to be sold

Selling the family home to fund care is sometimes the right answer and almost always done too early. Before you sell, price three alternatives: a reverse mortgage if the parent is 60 or older and the property is self-occupied; renting out one floor while the parent keeps the ground floor; and a loan against the property rather than an outright sale. Each keeps the asset and produces cash. None is free, and all three are worth a conversation with the bank before the deed is drafted.

If you do sell, remember that the parent's principal residence has capital-gains treatment that differs from an investment property, and that reinvestment windows are time-bound. Get a chartered accountant's view before signing, not after.

CHAPTER TEN

The law on your side

India has a specific law for exactly this situation. It is fast, it is cheap, and almost nobody uses it. Here is how it works, and where it stops.

The Maintenance and Welfare of Parents and Senior Citizens Act, 2007 is the only statute in India written specifically for the relationship between an older person and their adult children. It is deliberately informal: no lawyer is required, there is no court fee, and the forum sits at the sub-district level.

It is also widely misunderstood, partly because a 2019 amendment bill that would have expanded it was introduced, referred to a Standing Committee which reported in January 2021, and then lapsed. It is not law. Any advice that relies on the 2019 changes is wrong.

Who can file, and against whom

A senior citizen, or a parent, who is unable to maintain themselves from their own earnings or property can apply. So can a relative on their behalf. The obligation falls on children and, in some circumstances, grandchildren, and the definition of children in the 2007 Act is narrower than the lapsed bill's would have been: step-children, adoptive children and children-in-law were among the categories the 2019 bill sought to add.

The forum and the timeline

Element	Rule under the 2007 Act
Forum	Maintenance Tribunal, usually presided over by the Sub-Divisional Magistrate of the area where the parent lives
Cost	No court fee; a lawyer is not required
Order	Monthly allowance, up to Rs 10,000 per month under section 9(2)
Timeline	Tribunal to decide within 90 days of serving notice, extendable once by 30 days in exceptional cases (section 5(4))
Interim relief	Interim maintenance can be ordered while the case is pending
If unpaid	Warrant and imprisonment of up to one month for each unpaid month (section 5(8))

Read the Rs 10,000 cap carefully, because it cuts both ways. It is the ceiling on what a tribunal can order, which makes the forum useless for a family fighting over Rs 40,000 a month of care costs. It is also the reason the forum is fast: the sums are small, the procedure is summary, and the tribunal is not conducting a civil trial.

The property clause that changes outcomes

Section 23 is the provision most families should know about and almost none do. Where a senior citizen has transferred property to a child or relative on the condition that the transferee will provide maintenance, and the transferee then fails to provide it, the tribunal can declare that transfer void. The property returns to the parent.

The practical implication: if a parent has already signed a house over to a son or daughter in exchange for care, and the care has stopped, there is a statutory route to unwind it. It requires

evidence of the condition, so any written record of the arrangement, even a letter or a WhatsApp message, matters enormously.

Section 24 deals separately with abandonment, making it an offence to leave a senior citizen in a position where they cannot maintain themselves.

How a filing actually runs

1. The parent files an application with the Maintenance Tribunal of the sub-division where they live. Many districts have a simple form at the SDM office; a plain application setting out the facts is accepted.
2. Attach the relationship proof (ration card, Aadhaar, family records), the parent's income and asset position, and an estimate of monthly needs.
3. The tribunal issues notice to the children and attempts conciliation first. A large share of cases end here, with an agreed amount.
4. If conciliation fails, the tribunal hears the parties, usually within the 90-day window, and passes a reasoned order.
5. Non-compliance triggers the warrant and imprisonment provision, which is what gives the order teeth.

Where the 2007 Act does not help

It does not divide property, decide inheritance, or settle a title dispute. It does not award damages. It does not touch a dispute between siblings. And it cannot order more than Rs 10,000 a month. For anything larger, or for a property claim, you are in a civil court, and the timescale is years, which is exactly why the family agreement templates in chapter fourteen exist.

The other legal instruments, briefly

Three documents do most of the work in ordinary families, and they are routinely confused with each other.

Document	What it does	What it does not do
Will	Says who gets what after death; can be changed any time	Does not take effect during life; does not avoid probate questions
Power of attorney	Lets a named person act on the parent's behalf	Ends at death, can be revoked, and banks often refuse a general POA for their own risk reasons
Bank mandate or joint holder	Lets a family member operate the account in practice	A joint holder becomes an owner of the money, which has succession consequences

The bank mandate is the one to think hardest about. Adding a child as a joint holder is convenient and, on death, the money passes to the survivor by survivorship rather than under the will, which can defeat the parent's intentions for the other siblings. A mandate or an authorised signatory arrangement achieves the convenience without the ownership change. Ask the branch for a mandate, not a joint account.

On wills, one paragraph of practical advice. An unregistered will, written clearly, signed and attested by two witnesses, is valid in India. Registration adds evidence of date but is not required. What kills wills in practice is ambiguity, not formality: 'my house to my son' when there are two houses, or no residue clause for what is left over. Get a professional to draft it once, and review it after any birth,

death, marriage or major purchase.

This chapter is general information about how the law is structured. It is not legal advice for your situation, and where money or property is significant, a local advocate's hour is the cheapest part of the process.

CHAPTER ELEVEN

The benefit stack: what they are entitled to

Six schemes, one helpline and a renamed pension programme most articles still describe with the old name. Claim all of it; it takes a morning.

Government benefits for older Indians are spread across four ministries, which is why nobody has a single list. Here it is, current to November 2026, with what each one actually pays and how to start.

The pension programme changed its name in October 2026

On 14 October 2026 the Union Cabinet approved continuation of the National Social Assistance Programme for five years, 2026-27 to 2030-31, with an outlay of Rs 53,428 crore, and renamed the programme and each of its sub-schemes. This matters practically, because the old names are still printed on forms, and office staff may use either.

Old name	New name from October 2026
National Social Assistance Programme (NSAP)	Pradhan Mantri Samajik Sahayata Yojana
Indira Gandhi National Old Age Pension Scheme (IGNOAPS)	Pradhan Mantri Vridhavastha Pension Yojana
Indira Gandhi National Widow Pension Scheme (IGNWPS)	Pradhan Mantri Vidhawa Pension Yojana
Indira Gandhi National Disability Pension Scheme (IGNDPS)	Pradhan Mantri Divyangjan Pension Yojana
National Family Benefit Scheme	Pradhan Mantri Parivar Labh Yojana

The old age pension continues in its existing form: a central share of Rs 200 a month for ages 60 to 79 and Rs 500 a month from 80, for households below the poverty line, with states adding their own top-up on top. The disability pension was the one that changed, with the benchmark disability threshold dropping from 80 per cent to 40 per cent, eligibility running from birth, and central assistance rising to Rs 375 a month up to age 79 and Rs 625 from 80. Total coverage is now 3.20 crore beneficiaries across all 36 states and union territories.

The state top-up is where the money actually is. Kerala pays a state social security pension of Rs 1,600 a month across categories, on top of the central share, a top-up of Rs 1,100 to Rs 1,400 per pensioner. Reported state totals for 2026 include roughly Rs 3,200 in Haryana, Rs 2,500 in Maharashtra, Rs 1,500 in Punjab and Uttarakhand, Rs 1,450 in Rajasthan, Rs 1,200 in Tamil Nadu, Rs 1,100 in Bihar, Rs 1,000 in Uttar Pradesh and Jharkhand, and Rs 600 in Madhya Pradesh. Delhi's figure is reported variously between Rs 2,000 and Rs 3,000 depending on age band and source. Treat all of these as indicative and confirm at the state portal, because they change with every state budget.

How to apply, in practice

Apply at the Block Development Office in rural areas, or the municipal ward office or e-district portal in urban areas, under the state's social security pension scheme. You will need Aadhaar, age proof, a BPL card or the state's equivalent eligibility proof, a bank account in the beneficiary's own name with the Aadhaar linked, and a passport photo. Insist on an acknowledgement slip with a date. Follow up at 30 days; sanctions routinely stall without a follow-up.

Ayushman Bharat for the over-70s

Cover of Rs 5 lakh per year for every person aged 70 or above, irrespective of income, with the age taken from Aadhaar and Aadhaar-based e-KYC mandatory. Enrolment issues the Ayushman Vay Vandana card. Chapter four covers the detail and the limits. This is the single largest entitlement most families have not claimed.

Rashtriya Vayoshri Yojana: free assistive devices

Free physical aids and assisted-living devices for senior citizens from lower-income households, distributed by ALIMCO through assessment camps. The devices include walking sticks, walkers, wheelchairs, hearing aids, spectacles and dentures, depending on what the assessment finds.

- Eligibility: senior citizens from households with monthly income below Rs 15,000, certified by the tehsil, the village head, or a pension document.
- Documents: Aadhaar and the income certificate.
- How to start: register on the ARJUN portal, or approach the district social welfare office and ask when the next ALIMCO assessment camp is scheduled for your district.
- Why it matters: a wheelchair and a walker bought privately cost Rs 6,000 to Rs 15,000. The camp is free and the assessment is done by a professional.

Atal Vayo Abhyuday Yojana and Elderline 14567

AVYAY is the umbrella programme for senior citizen welfare, and it funds the national helpline, senior citizen homes, continuous care homes, mobile medicare units and physiotherapy centres. The helpline is the part every family should know.

Elderline 14567 was launched on 1 October 2021 and is operational in 32 states and union territories. By September 2026 it had received over 29 lakh calls, of which more than 8.67 lakh were actionable, covering information, guidance, emotional support and field intervention. It is toll-free.

Use it for three things specifically: guidance on a scheme or entitlement you cannot resolve locally, emotional support for an isolated parent who will talk to a stranger more readily than to family, and field intervention where a senior citizen is being neglected or abused and the family needs an official record. Register the parent's number and address with the helpline before you need it.

Health programmes and the rest

Benefit	What it gives	Where to start
National Programme for Health Care of the Elderly	Dedicated geriatric services at district hospitals and below, including physiotherapy and home-based care	The district hospital's geriatric or NPHCE cell
Senior Citizens' Welfare Fund	Interest income from unclaimed deposits, used for senior citizen welfare programmes	No individual claim; funds AVYAY activities
Income tax deductions	Section 153 up to Rs 50,000 interest, 126 up to Rs 50,000 medical premium, 128 up to Rs 1,00,000 for specified diseases	Chapter five; only under the old regime
Maintenance Tribunal	Up to Rs 10,000 a month from children who refuse support	The SDM office of the parent's sub-division
Concessions	Rail fare concessions for senior citizens, priority queues, reserved counters at banks and hospitals	Carry Aadhaar or a senior citizen card

One caution about rail concessions. The fare concession for senior citizens was withdrawn during the pandemic and has not been universally restored, so check the current IRCTC position before promising it to a parent. Priority queues and reserved counters at banks, hospitals and government offices are separate and remain in place.

What to do this week

1. If the parent is 70 or older, check whether an Ayushman Vay Vandana card exists. If not, enrol this week.
2. Check whether they receive a state old age pension. If not, establish eligibility at the block or ward office.
3. If they use a walker, stick, wheelchair or hearing aid that was bought privately, register on ARJUN for the next assessment camp.
4. Save 14567 in the parent's phone under a name they will recognise, and in yours.

CHAPTER TWELVE

Banks, pensions and the digital estate

This month, every pensioner in India has a deadline. Beyond it, four small administrative acts prevent the worst week of a family's year.

November is life-certificate month. Pensioners aged 80 and above could start submitting from 1 October; everyone else submits between 1 and 30 November. Miss it and the pension stops, which then takes weeks to restart and is exactly the kind of administrative failure that lands on the family at the worst time.

Jeevan Pramaan: the ten-minute version

Jeevan Pramaan generates a Digital Life Certificate through Aadhaar-based face authentication on a smartphone. No branch visit, no attestation by a gazetted officer, no queue.

1. Install the Jeevan Pramaan face-authentication app on your phone, not the parent's, if their phone is old.
2. Have the parent's Aadhaar number and the pension account details ready: bank name, account number and the PPO number for government pensioners.
3. Sit the parent facing a window in daylight, remove spectacles if the app asks, and follow the on-screen face capture.
4. Complete the Aadhaar OTP step, and save the generated certificate number.
5. Forward the certificate number to the bank or pension office if the system does not push it automatically, and keep a screenshot in the eldercare file.

If face authentication fails, three fallbacks exist: the bank branch, which must accept a physical life certificate; a doorstep visit, which most banks now offer for immobile customers; and the India Post Payments Bank postman, who can generate a digital life certificate at the door.

The nomination audit, which takes twenty minutes and saves months

RBI's directions on settlement of claims for deceased customers, issued in September 2025, standardised what banks must do and how quickly. But the single biggest determinant of how smoothly a claim settles is whether a nomination was registered. Banks are required to settle claims within a defined timeline, and delays beyond it attract compensation at the Bank Rate plus 4 per cent. None of that helps if the paperwork is missing.

So do the audit. For every account, deposit, locker and demat holding, check that a nominee is registered, and that the nominee is the right person. Nomination is not inheritance; it decides who receives the money, and it overrides the will for that specific asset. Where the parent's intent is equal division among three children, a single nominee creates a problem the family will have to solve with goodwill.

Asset	What to check	Where
Savings accounts	Nominee registered and correct	Branch or net banking
Fixed deposits	Nominee on each deposit, not just the account	Branch
PPF, NPS, SCSS	Nominee on the scheme itself	Post office, bank, NPS portal

Asset	What to check	Where
Mutual funds	Nominee at folio level, or 'not nominated' flagged	AMC or RTA, or the MF Central portal
Demat	Nominee at the depository participant	DP
Insurance	Nominee, and whether it is a minor	Insurer
Locker	Nominee and the locker agreement	Branch
EPF or pension	EPS nomination Form 2 / 11	EPFO member portal

Unclaimed money: check this once

Billions of rupees sit in unclaimed deposits and shares because families did not know an account existed. Two free searches settle it. RBI's UDGAM portal searches unclaimed deposits across participating banks by name and PAN. The IEPF authority's portal does the same for unclaimed dividends and shares. Run both with the parent's details; it takes minutes and occasionally turns up a forgotten fixed deposit from 1998.

The digital estate

Your parent probably has a phone with UPI, a Gmail account, a WhatsApp account and perhaps an insurance app. That is a financial estate with no paper trail, and it is the part families are least prepared for.

- Write down which apps hold money: UPI, wallets, any investment app.
- Note the phone's lock method, and whether a family member can unlock it. Do not store the PIN in the same place as the phone.
- Check that the phone number linked to Aadhaar, bank accounts and demat is one the family can access. A disconnected number is the single most common blocker in every claim process.
- Keep the email account recoverable. Almost every financial account resets through email, and an inaccessible email locks the whole estate.
- For accounts with a legacy or nominee setting, use it. Google and Apple both offer one; it is the difference between access and a court order.

The password rule that actually works

Do not keep passwords in a notebook in the same drawer as the passbook. Use a shared password manager with an emergency-access feature, or a sealed envelope with a trusted third person, or simply a written list held by the sibling who does not handle the money. The point is that two people, not one, can open the door.

If the parent is still working or earning

A growing number of Indians over 60 are still earning: NITI Aayog's analysis found only about a third of the elderly are currently working, and for many of them the income is informal. Two administrative points follow. Interest income must be declared, and Form 121 replaces the old 15G and 15H declarations under the Income-tax Act 2025. And where the parent is a landlord, rent should be received into a bank account rather than in cash, for both tax and succession reasons.

CHAPTER THIRTEEN

The family: siblings, hard conversations, your limits

Most eldercare collapses are not medical or financial. They are relational. Here is how to structure the family part so it does not become the problem.

The pattern is remarkably consistent. One child, usually the daughter or the unmarried son, or the one who lives closest, becomes the default caregiver. The others contribute money, opinions, or nothing. Ten years later there is a property dispute, and the real subject of the dispute is the ten years.

It does not have to go that way, but it does not fix itself either. Three structures do most of the work: a named division of labour, a money agreement, and a scheduled conversation.

The division of labour that holds

Do not divide care by willingness. Divide it by function, and write the functions down with names against them. Six functions cover almost everything.

Function	What it involves	Realistic owner
Daily presence	Calls, visits, noticing changes	Whoever lives closest, or a paid attendant with family oversight
Medical coordination	Appointments, discharge follow-up, the medical sheet	Whoever is best at institutions, not necessarily the closest
Money	Bills, insurance claims, pension, budget	The sibling who is numerically careful; never the same person as daily care
Paperwork and law	Documents, banks, property, will	Whoever has the patience for queues
Emergency cover	The 2 a.m. call, hospital admission, travel	Rotate quarterly so one person is not always on call
Respite	Taking over for a fixed block so the primary carer rests	Every sibling, on a schedule, non-negotiable

The separation of money from daily care is the single most useful structural decision. When the person doing the care also controls the money, every expense becomes a suspicion, and the caregiver has no defence that anyone can verify. Split them and most of the friction disappears.

The money agreement

Write it down, one page, all siblings sign it, and revisit it once a year. It needs four numbers and two rules.

- The monthly contribution per sibling, and the date it is transferred.
- Who holds and disburses it, and how it is accounted for: a shared spreadsheet, updated monthly, visible to all.
- What it covers, and what triggers a top-up: a hospital admission, a care upgrade, a new medicine regimen.

- The rule for spending above a stated threshold, which requires all siblings to be informed before, not after.
- The rule that hands-on care counts as a contribution, valued at the attendant wage it replaces.
- The rule that the parent is told what is being spent, because secrecy about their own money damages trust more than the money does.

The four conversations you are avoiding

Each of these is uncomfortable for a reason, and each is much worse if it happens during a crisis instead of before one.

1. Driving

Do not argue about competence; argue about evidence. Ask the parent to take you on a short drive as a passenger and observe: lane discipline, reaction at a junction, night vision, whether they check the mirror. If you see three or more lapses, propose a driving assessment rather than a ban. An occupational therapist or a driving school can do one, and a third party's verdict is accepted far more readily than a child's.

2. Money and the will

Frame it as reducing burden, not as anticipating death. 'If something happens to you, I do not want the family fighting in a bank queue. Can we spend an hour putting the nominations and the will in order?' Most parents respond to that framing because it is about protecting the children from each other, which is usually their actual concern.

3. Moving

Never present a move as a decision already taken. Present it as a question with a deadline: 'We need to decide by March whether the stairs are manageable. Here are three options, including staying.' A parent who feels in control of the decision will often choose the option you think is right; a parent who feels managed will refuse everything.

4. What they want at the end

This is the conversation nobody has, and it is the one that determines whether the last months are spent in a hospital or at home. Ask three questions, once, when everyone is well: 'If you became very ill and could not speak for yourself, who should decide, and what would you want them to know?' 'Would you want to be at home if it were possible?' 'Is there anything you want said or done that we should not leave to chance?' Write the answers down and share them with every sibling. Chapter fifteen turns this into a document.

Your own limits, stated plainly

Caregiver burnout is not a metaphor. The symptoms are specific: sleep that does not restore, irritability at small things, a sense of resentment you feel guilty about, and the quiet wish that something would happen to end the situation. If you recognise three of those, you are not failing; you are overloaded, and overload is a design problem with a design solution.

Four things work, in this order. Respite that is scheduled rather than requested, so it actually happens. Delegation of one whole function, not bits of several. A peer group of people in the same situation, which is worth more than sympathy from people who are not. And a professional if the low mood persists for more than a few weeks, because depression in caregivers is common, treatable and rarely self-diagnosed.

The resentment is information, not a character flaw

If you are the only sibling doing the work, the resentment is telling you the structure is unfair. Fix the structure, not the feeling. Say the sentence out loud: 'I am doing X hours a week and I need it to be Y.' Specific, numeric, and addressed to the people who can change it. Most siblings are not refusing to help; they are waiting to be told exactly what help looks like.

When it is abuse

Elder abuse in India is under-reported and mostly happens inside the home. The signs are physical (unexplained bruises, dehydration, bedsores in someone who is not bedridden), financial (sudden changes to accounts, missing jewellery, a new 'friend' managing money), and psychological (sudden fear of a particular person, withdrawal, a change in the will after a new acquaintance appeared).

You have three levers. The Maintenance Tribunal under the 2007 Act, described in chapter ten. Elderline 14567, which can record a complaint and trigger field intervention. And the police, where there is a cognisable offence, including under section 24 of the Act for abandonment. Document before you confront: dates, photographs, bank statements, and a written note of what the parent said, in their words.

CHAPTER FOURTEEN

Templates and checklists

Everything in this book that fits on a page, on a page. Print these, fill them in, and stop relying on memory.

These are working documents, not legal instruments. Use them as written, adapt the numbers, and where money or property is significant, have a professional review the final version.

Template 1: the emergency card

Wallet-sized. One copy in the parent's purse, one in each child's wallet, one photographed on every sibling's phone.

Field	Fill in
Name and date of birth	
Blood group	
Conditions	
Medicines and doses	
Allergies	
Insurance: policy number and TPA toll-free	
Ayushman card number	
Family doctor and phone	
Son or daughter 1 and phone	
Son or daughter 2 and phone	
Nearest hospital and distance	

Template 2: the sibling care agreement

One page, signed by every sibling, reviewed annually.

- We agree that our parent's care will be divided by function as set out below, with a named owner for each function.
- Daily presence: ____. Medical coordination: ____. Money and claims: ____. Paperwork and law: ____. Emergency cover, rotating quarterly: ____.
- Each sibling contributes Rs ____ per month, transferred by the ____ of the month to the care account held by ____.
- Expenses above Rs ____ require all siblings to be informed before they are committed.
- Hands-on care is a contribution valued at Rs ____ per month, being the attendant wage it replaces.
- A shared spreadsheet records every expense and is updated monthly.
- Respite: each sibling takes a fixed block of ____ days per quarter so the primary carer is relieved.
- This agreement is reviewed every ____ and may be changed in writing with everyone's consent.
- Signatures and dates.

Template 3: the attendant engagement letter

- Parties: the family, represented by ____, and ____ (the attendant), Aadhaar ____.
- Duties: the specific list of care tasks, written out, plus household tasks if any.
- Hours: ____ per day, and the night arrangement stated explicitly.
- Salary: Rs ____ per month, paid by bank transfer on the ____ of each month.
- Included: food and accommodation, or a stated allowance instead.
- Days off: ____ per month, with ____ providing cover.
- Notice: ____ days on either side.
- Medicines: no change may be made except on the instruction of the family or a doctor.
- Money: no access to the parent's bank account, card, UPI or pension. Any cash for household purchases is recorded in a book.
- Emergency: call an ambulance first, then the family, in that order.
- Confidentiality: the parent's medical and financial information is not discussed outside the household.
- Signatures and dates, and a copy given to the attendant.

Template 4: the monthly care review

Ten minutes, once a month, same three questions, logged in one place.

Question	This month's answer	Action and owner
What changed in health, mood or mobility?		
What is running out or falling behind?		
What does the parent want that is not happening?		
Actual spend vs budget		
Any safety incident		

Template 5: the hospital admission checklist

1. Insurance card, Ayushman card, Aadhaar, and the policy number written on a slip.
2. Medical sheet, current medicine strips, and the last two discharge summaries.
3. Pre-authorisation filed before a planned admission; ask for the reference number and write it down.
4. Ask who the treating consultant is and when the daily round happens.
5. Ask what is not covered by the policy on this admission.
6. Keep every bill, receipt and investigation report in one envelope from day one.
7. Before discharge, get the summary in hand and reconcile the medicine list against what you are actually given.
8. Ask the one question: what is most likely to go wrong in the next two weeks, and what should make us come back immediately.

Template 6: the home safety audit

Area	Check	Done
Bathroom	Grab bar by toilet and shower; anti-skid mat; stool; no high threshold	
Bedroom	Light reachable from bed; clear path to toilet; firm mattress	
Stairs	Rail on both sides; good lighting at top and bottom; nothing stored on steps	
Kitchen	Gas knob reachable and safe; heavy items at waist height; no stool climbing	
Floor	No loose rugs; no cables across walking routes; dry	
Footwear	Sole intact; back strap; not loose slippers	
Medicines	Weekly organiser filled; one named person; expired strips removed	
Emergency	Phone charged and reachable; family numbers on speed dial; 14567 saved	
Documents	Aadhaar, insurance, medical sheet in one labelled envelope	

Template 7: the quarterly money check

- Three months of bank statements read, not skimmed, for unusual debits.
- Insurance premium paid, and the receipt filed.
- Fixed deposits and SCSS maturities noted for the next two quarters.
- Form 121 submitted at every branch where interest may exceed the threshold.
- Pension credited every month without a break; life certificate submitted on time.
- Care budget compared to actual, and adjusted.
- The eldercare file reviewed and updated with any new diagnosis or policy.

Seven templates, twenty minutes each, four times a year. That is the entire administrative load of eldercare once the system is built, and it is the difference between a family that copes and a family that is permanently firefighting.

CHAPTER FIFTEEN

The last chapter, and the thirty days after

The documents to have ready, the directive that is legally recognised, and the administrative checklist nobody hands you at the crematorium.

This chapter is written plainly because euphemism makes it useless. If a parent is very old or very ill, the practical work of the last months is mostly paperwork and presence, and doing the paperwork early is how you buy the presence.

The advance medical directive

India recognises advance medical directives, commonly called living wills, following the Supreme Court's judgment in Common Cause v Union of India in 2018, as modified in 2023. The 2023 modification simplified the process considerably: the requirement of judicial magistrates' countersignature was removed, and notarisation by a notary or a gazetted officer, with the directive attested by two witnesses, is what is now required.

A directive does two things. It records what the person would want if they become unable to decide, and it names who should decide for them. Both matter, because the second is the part families fight about.

What a directive should say, in five lines

1. Who I am, and that I am of sound mind as I write this.
2. The treatments I would refuse if there is no reasonable prospect of recovery, described in ordinary words, not clinical terms.
3. The treatments I would want, including pain relief, even if it hastens death as a side effect.
4. The person I name to decide, and a substitute if they are unavailable.
5. Signature, date, two witnesses, and notarisation.

Two practical warnings. A directive is not a do-not-resuscitate order and does not override a doctor's clinical judgement; it informs it. And most Indian hospitals are unfamiliar with the instrument, so the family should carry a copy and be prepared to explain it calmly. Where palliative care is the goal rather than cure, ask for a palliative care referral early. India's National Programme for Palliative Care has expanded community-based services, and home-based palliative care transforms the last months for both the patient and the family.

The documents to have ready before you need them

- Aadhaar, PAN and voter ID, in one envelope, with photocopies.
- The will, and the name and number of whoever holds it.
- Bank account details, and the nomination status of each.
- Insurance policies, with the claim helpline numbers.
- Pension documents, PPO number, and the pension office contact.
- Property papers, and the municipal tax receipts.
- The names, addresses and phone numbers of the immediate family, written down, because a phone is not always available.
- Any organ donation pledge, if there is one, and the family's knowledge of it.

In the first hours

In an urban setting, a death at home requires a medical certificate of cause of death, obtained from a doctor or the municipal authority, before the body can be moved or cremated. If the death is at a hospital, the hospital issues it. If the death is at home and the family doctor is unavailable, the municipal health officer or the local police station will guide you. Register the death within the statutory window, and get multiple certified copies of the death certificate; you will need between eight and fifteen.

Then, and only then, deal with the administrative estate. Nothing below is urgent within twenty-four hours, and doing it while exhausted produces errors.

The thirty-day administrative checklist

Days	Action	Where
1-7	Death certificate, 10-15 certified copies	Municipal office or hospital
1-7	Inform the pension office and stop the pension; a pension credited after death must be returned	Bank or pension disbursing office
1-7	Inform the employer if the parent was receiving a family pension	Employer or treasury
7-15	Bank accounts: lodge the claim with the nominee and the death certificate; banks must settle within the timeline set by RBI's 2025 directions	Branch
7-15	Insurance claims: life and health, with the death certificate and policy documents	Insurer or TPA
15-30	Mutual funds, demat and PPF: transmission to the nominee or legal heir	AMC, DP, post office
15-30	Property: mutation of municipal records and the revenue record	Municipal office, tehsil
15-30	Close or transfer utilities, phone, and any subscriptions	Providers
30-90	Income tax return for the year, filed by the legal representative	Tax portal
As needed	Probate or succession certificate if there is no nominee and no will, or a dispute	Civil court

Two things shorten this list dramatically. A registered nomination on every asset, and a clear will. Families that have both finish the estate in weeks; families with neither spend a year in a civil court and pay a lawyer more than the asset was worth.

The part that is not administrative

After the paperwork, there is a year in which the family discovers what the parent was actually holding together. Two things help. Keep the routines that involved them: the Sunday call, the Diwali visit, the walk. And give the primary caregiver, who is often the most exhausted and the least mourned, explicit permission to stop, because the habit of care does not switch off when the need ends.

If the last months were hard, the family will replay them. The single most useful thing you can say to a sibling in that year is not 'you did enough' but something specific: 'You were there on the Thursday when I could not be, and that mattered.' Specificity is what makes it land.

That is the end of the manual. The rest of the book is sources, so that you can check anything here against the document it came from.

APPENDIX

Sources and how to check them

Every figure in this edition traces to one of these. Where a number will move, the primary source is listed so you can check the current value.

This edition is dated November 2026. Rates, scheme names and thresholds change; where they do, the primary source below is the place to look, not a news summary of it.

Population, ageing and health

- NITI Aayog, *Senior Care Reforms in India: Reimagining the Impact Agenda*, February 2024. The 104 million and 319 million projections, the one-in-five-by-2050 framing, and the finding that only about a third of the elderly are currently working.
- HelpAge India, *Salient Findings*, 2026. Survey of 2,224 older persons across 20 districts in 10 states: 46% with a long-term impairment, 32% mobility, 24% vision, 48% financially dependent, 92% with personal monthly income below Rs 10,000, 13% living alone.
- Longitudinal Ageing Study in India (LASI), India Report 2020, International Institute for Population Sciences.
- National estimates of dementia prevalence derived from LASI, published in *Alzheimer's and Dementia* (2023): 7.4% prevalence among adults 60+, an estimated 8.8 million Indians over 60 living with dementia.
- Outlook Money, citing NITI Aayog: 75% of senior citizens in India live with at least one chronic disease.

Health financing and insurance

- National Health Authority, *FAQs related to the benefits for senior citizens under the AB-PMJAY scheme*: all persons aged 70+, irrespective of income, Rs 5 lakh cover, approximately 6 crore seniors in about 4.5 crore families, Aadhaar e-KYC mandatory.
- Press Information Bureau and PMO, Cabinet approval of health coverage for all senior citizens aged 70 and above, October 2024.
- IRDAI Master Circular on health insurance, 29 May 2024: offer of products to all ages and to persons with pre-existing disease, pre-existing disease waiting period reduced to three years, moratorium period reduced to five years.

Money, tax and pensions

- Department of Economic Affairs, office memorandum of 30 September 2026: small savings rates for October to December 2026, including SCSS at 8.2%.
- National Savings Institute and India Post, SCSS scheme terms: Rs 1,000 to Rs 30 lakh, five-year term with one three-year extension, quarterly interest, eligibility from age 60 or 55 on retirement.
- Income-tax Act 2025, renumbered deduction provisions: section 153 for interest income (formerly 80TTB), section 126 for health insurance and medical expenditure (formerly 80D), section 128 for specified diseases (formerly 80DDB), section 123 for the investments formerly under 80C. Form 121 replaces Forms 15G and 15H.

- Press Information Bureau, 14 October 2026: Cabinet approval of the continuation of the National Social Assistance Programme for 2026-27 to 2030-31, renamed Pradhan Mantri Samajik Sahayata Yojana, with IGNOAPS renamed Pradhan Mantri Vridhavastha Pension Yojana; outlay Rs 53,428 crore; 3.20 crore beneficiaries across 36 states and union territories; old age and widow pensions continue in their existing form.
- Ministry of Rural Development, NSAP guidelines: central assistance of Rs 200 a month for ages 60-79 and Rs 500 from 80 under the old age pension, for below-poverty-line households.
- Kerala Local Self Government Department, NSAP page: state social security pension of Rs 1,600 a month, a top-up of Rs 1,100 to Rs 1,400 per pensioner.

Law and banking

- Maintenance and Welfare of Parents and Senior Citizens Act, 2007, sections 4, 5, 9, 23 and 24; India Code, Ministry of Law and Justice.
- The Maintenance and Welfare of Parents and Senior Citizens (Amendment) Bill, 2019: introduced 11 December 2019, Standing Committee report 29 January 2021, lapsed. Not law.
- Reserve Bank of India, *Settlement of Claims in respect of Deceased Customers of Banks) Directions, 2025*, RBI/2025-26/82, 26 September 2025.
- Supreme Court of India, *Common Cause v Union of India*, 2018, as modified in 2023, on advance medical directives.

Schemes and helplines

- Press Information Bureau, 23 September 2026: Elderline 14567, launched 1 October 2021, operational in 32 states and union territories, over 29 lakh calls and 8.67 lakh actionable calls to September 2026.
- ALIMCO, Rashtriya Vayoshri Yojana FAQ and scheme document: income below Rs 15,000 a month, Aadhaar and income certificate, assessment camps, registration through the ARJUN portal.
- Department of Social Justice and Empowerment, Atal Vayo Abhyuday Yojana programme documents.
- Department of Pension and Pensioners' Welfare, Jeevan Pramaan: submission window from 1 October for pensioners aged 80 and above, and 1-30 November for others.

Care costs

- Industry cost guides published in 2026 by home-care providers operating in Delhi, Mumbai, Bangalore and Hyderabad: attendants at Rs 12,000-25,000 a month live-in and Rs 150-300 an hour part-time; trained nurses at Rs 25,000-60,000 a month and Rs 300-800 an hour; assisted living at Rs 25,000-80,000 a month, with premium communities above Rs 1 lakh. These are commercial quotes, not regulated tariffs, and they vary by city and by the acuity of the patient.

A note on what this book is not

This is a planning and administration manual. It is not medical advice, legal advice or financial advice for your specific circumstances, and it does not replace a doctor, an advocate or a chartered accountant. Where a decision is consequential, the correct use of this book is to arrive at that professional's office already knowing what to ask.

The Indian Eldercare Playbook 2026-27. First edition, November 2026. Prepared for readers in India; all amounts in Indian rupees.